AR-03 Drug-Assisted Airway

CLAIBORNE COUNTY EMS | MEDICAL-DIRECTOR APPROVED | SOURCE PDF | OCTOBER 17, 2026

Scope: Optional high-risk Paramedic procedure for airway protection failure, inadequate oxygenation/ventilation, or impending compromise when less-invasive care is inadequate. Requires agency authorization, documented competency, performance improvement, and at least two Paramedics on scene.

Field flow

1. Prepare	Two Paramedics, monitoring, suction, BVM/PEEP, primary and rescue airway, and surgical-airway plan.
2. Preoxygenate	High-flow oxygen, NIPPV when appropriate, or two-person BVM/PEEP. Correct hypoxia and hypotension first.
3. Medicate	Select one induction agent and the appropriate paralytic; proceed immediately to placement.
4. Confirm	Waveform EtCO2, bilateral exam, tube depth, SpO2, securement. If ineffective: BVM/BIAD and AR-02.

Provider-level actions

Level	Approved actions
EMT	Assist with preoxygenation, suction, BVM, equipment preparation, monitoring, and backup-airway readiness.
AEMT	All EMT care plus authorized vascular access, rescue-BIAD support, monitoring, and failed-airway-plan assistance.
Paramedic	Verify indication, authorization, staffing, rescue capability, resuscitation, medications, and post-airway pain-first care before medication.

AR-03 Drug-Assisted Airway

SAFETY, MEDICATION, AND LINKED-PATHWAY REFERENCE

Safety and escalation

• Do not perform DAI for convenience, merely because BVM is effective, or for a short transport when BVM/BIAD safely maintains oxygenation and ventilation.

• Limit laryngoscopy to three total attempts; each later attempt must change the provider, approach, position, or equipment.

• Do not routinely repeat rocuronium before post-intubation analgesia and sedation are established.

• Pediatric use requires a direct online order from the Claiborne EMS Medical Director or Assistant Medical Director.

Medication reference

Medication	Approved dose / safeguard
Ketamine	1-2 mg/kg IV/IO; if no access: 4 mg/kg IM, max 400 mg
Etomidate	0.3 mg/kg IV/IO
Succinylcholine	1-2 mg/kg IV for RSI
Rocuronium	1 mg/kg IV/IO when indicated
Fentanyl	25-50 mcg IV/IO; repeat cautiously, max 100 mcg
Midazolam	2-5 mg IV/IO; repeat cautiously after analgesia

Linked pathways

AM-05 Hypotension / Shock; AM-07 Crashing Medical Patient; AR-02 Adult Failed Airway; AR-08 Post-Intubation / BIAD Management; Push-Dose Epinephrine reference.

Source control

• This controlled direct source PDF replaces the prior source file and matches approved Claiborne EMS app content released October 17, 2026.

• Use current Claiborne standing orders, formulary, equipment authorization, and Medical Control direction whenever they govern a case-specific decision.